MATERNAL & NEWBORN HEALTH NURSING · LECTURE 8 · QUESTION BANK

Assessment of Fetal Well-being

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28	8	15
Past exam questions	NCLEX-style	Flashcards

المصادر اللي اتجمعت منها أسئلة المحاضرة دي:

أسئلة سابقة · كويز 2 – 2021 (نسخة أ) · كويز 2 – 2021 (نسخة ب) · كويز 2 – 2022 · ميدترم 2022–2023

جنب كل سؤال مصدره. لو السؤال اتكرر في كذا امتحان مكتوب كل مرة بمصدرها، وعليه علامة "تكرر". الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

Part A · Past Exam Questions

أسئلة الدفعات السابقة

1 ميدترم 2022–2023

The nurse has performed a non-stress test and interprets it as reactive. The nurse documents this finding as:

a. Normal
b. Abnormal
c. Need for further evaluation
d. Findings were difficult to interpret

2 ميدترم 2022–2023

A pregnant client is scheduled for a biophysical profile. The test measures:

a. Mother's ability to tolerate pregnancy
b. Amniotic fluid volume and fetal activity
c. Mother's cardiac status and ability to tolerate labor
d. Only the amount of amniotic fluid

3 ميدترم 2022–2023

A biophysical profile score of 6 or below indicates fetal compromise. (True / False)

a. True
b. False

4 ميدترم 2022–2023

The best description of the contraction stress test is:

a. The uterus is stimulated to contract by small amounts of oxytocin or nipple stimulation
b. The woman ambulates until contractions begin
c. Uterine contractions are stimulated by Leopold's maneuvers
d. Uterine contractions before 28 weeks of gestation

5 ميدترم 2022–2023

On assessment of a mother at 38 weeks the fetal heart rate is 174 bpm. What is the best action?

a. Document the finding
b. Check the mother's heart rate
c. Notify the health-care provider
d. Tell the client the rate is normal

6 The nurse auscultates a fetal heart rate of 145 bpm in a mother in early labor. Which action is appropriate? كويز 2 – 2022

a. Inform the mother that the rate is normal
b. Reassess in 5 minutes to verify the results
c. Immediately report the rate to the health-care practitioner
d. Place her on her left side and apply oxygen

7 The nurse auscultates a fetal heart rate of 152 bpm in a client in early labor. Which action is appropriate? كويز 2 – 2021 (نسخة ب)

a. Inform the mother that the rate is normal
b. Reassess in 5 minutes to verify the results
c. Immediately report the rate to the health-care practitioner
d. Place her on her left side and apply oxygen by face mask

8 Bradycardia refers to the average FHR during a 10-minute period that excludes periodic and episodic changes and periods of marked variability. (True / False) كويز 2 – 2021 (نسخة ب)

a. True **b.** False

9 Acceleration is a baseline FHR below 110 beats/min. (True / False) كويز 2 – 2021 (نسخة ب)

a. True **b.** False

10 Hypoxia is defined as insufficient availability of oxygen to meet the metabolic needs of body tissue. (True / False) كويز 2 – 2021 (نسخة ب)

a. True **b.** False

11 The onset, nadir and recovery of the deceleration occur after the beginning, peak and end of the uterine contraction in late deceleration. (True / False) كويز 2 – 2021 (نسخة أ)

a. True **b.** False

12 A pregnant woman asks the nurse to describe a contraction stress test. What should the nurse tell her? أسئلة سابقة

a. Uterine contractions are stimulated by small amounts of oxytocin
b. An external fetal monitor is attached and the woman ambulates until contractions begin
c. Uterine contractions are stimulated by Leopold's maneuvers
d. Uterine contractions are stimulated before 28 weeks of gestation

13 What is the priority nursing action when the fetal heart rate is 174 beats/minute? أسئلة سابقة

a. Document the finding
b. Notify the health care provider
c. Tell her the fetal heart rate is normal
d. Check the mother's heart rate

14 Which of the following is a characteristic of amniotic fluid? أسئلة سابقة

a. Can be used to measure fetal liver function
b. Prevents large particles such as bacteria from passing to the fetus
c. Provides exchange of nutrients and wastes between mother and fetus
d. Surrounds, cushions and protects the fetus

15 During the active stage of labor the nurse observes late decelerations on the monitor strip. What is the most appropriate nursing action? أسئلة سابقة

a. Administer oxygen via face mask
b. Place the mother in a supine position
c. Document the findings and continue to monitor
d. Increase the rate of the oxytocin infusion

16 At 38 weeks a client has late decelerations; the fetal pulse oximeter shows 75%–85%. The nurse should: أسئلة سابقة

a. Discontinue the catheter if the reading is not above 80%
b. Discontinue the catheter if the reading does not go below 30%
c. Advance the catheter until the reading is above 90%
d. Reposition the catheter, recheck, and if it is 55% keep monitoring

17 The nurse observes the client's amniotic fluid and decides it is normal because it is: أسئلة سابقة

a. Clear and dark amber
b. Milky, greenish-yellow with shreds of mucus
c. Clear, almost colorless, with little white specks
d. Cloudy, greenish-yellow with little white specks

18 After rupture of membranes the nurse notes variable decelerations in the fetal heart rate. The nurse should: أسئلة سابقة

a. Stop the oxytocin infusion
b. Change the client's position
c. Prepare for immediate delivery
d. Take the client's blood pressure

19 After Leopold's maneuvers the fetus is in ROP position. To best auscultate fetal heart tones, the Doppler is placed: أسئلة سابقة

a. Above the umbilicus at the midline
b. Above the umbilicus on the left side
c. Below the umbilicus on the right side
d. Below the umbilicus near the left groin

20 An elevation of 15 beats above a baseline of 135 bpm lasting 15 seconds should be documented as: أسئلة سابقة

a. An acceleration
b. An early elevation
c. A sonographic motion
d. A tachycardic heart rate

أسئلة سابقة

21 **For a client delivering twins, the nurse places the external fetal monitor:**

a. Over the most anterior fetus
b. Over the most posterior fetus
c. So that each fetal heart rate is monitored separately
d. On one fetus for 15 minutes then the other for 15 minutes

أسئلة سابقة

22 **After attaching an external electronic fetal monitor on admission to the labor room, the initial nursing assessment is:**

a. Identifying the types of accelerations
b. Assessing the baseline fetal heart rate
c. Determining the frequency of contractions
d. Determining the intensity of contractions

أسئلة سابقة

23 **The nurse suspects umbilical cord compression if which of the following is noted on the external monitor tracing during a contraction?**

a. Early decelerations
b. Variable decelerations
c. Late decelerations
d. Short-term variability

أسئلة سابقة

24 **A client in active labor has contractions every 3 minutes lasting 45 seconds; the fetal heart rate between contractions is 100 bpm. The most appropriate action is:**

a. Encourage the coach to continue breathing exercises
b. Encourage the client to continue pushing
c. Continue monitoring the fetal heart rate
d. Notify the physician or nurse-midwife

أسئلة سابقة

25 **The nurse notes episodic accelerations on the electronic fetal monitor tracing. The most appropriate action is:**

a. Document the findings and tell the mother the monitor indicates fetal well-being
b. Take maternal vital signs and tell her bed rest is required
c. Notify the physician or nurse-midwife
d. Reposition the mother and check the tracing for changes

أسئلة سابقة

26 **Using a Doppler ultrasound device, the nurse most accurately determines that fetal heart sounds are heard by:**

a. Noting if the heart rate is greater than 140 bpm
b. Placing the diaphragm of the Doppler on the abdomen
c. Performing Leopold's maneuvers first
d. Palpating the maternal radial pulse while listening to the fetal heart rate

أسئلة سابقة

27 **A client at 36 weeks is scheduled for amniocentesis. Which statement about the preceding ultrasound indicates a need for further teaching?**

a. The ultrasound locates the placenta
b. The ultrasound shows the blood flow through the umbilical cord
c. The test determines the needle insertion site
d. The ultrasound locates the amniotic fluid

28 What is a normal fetal heart rate at 38 weeks' gestation? أسئلة سابقة

a. 80 bpm
b. 100 bpm
c. 150 bpm
d. 180 bpm

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

N1 An NST shows two accelerations of 15 bpm lasting 15 seconds in 20 minutes. The nurse documents: NCLEX

a. Nonreactive
b. Reactive (reassuring)
c. Positive
d. Equivocal

N2 A BPP score of 4/10 indicates: NCLEX

a. Normal
b. Fetal compromise — notify provider
c. Equivocal, repeat next week
d. Maternal hypertension

N3 Late decelerations appear on the monitor. The FIRST nursing action is: NCLEX

a. Increase oxytocin
b. Turn the woman to her left side
c. Document and continue to monitor
d. Place her supine

N4 Select ALL components of the biophysical profile. SELECT ALL

a. Fetal breathing movements
b. Fetal body movements
c. Fetal tone
d. Amniotic fluid volume
e. Reactive NST
f. Maternal blood pressure

N5 Variable decelerations are caused by: NCLEX

a. Head compression
b. Uteroplacental insufficiency
c. Umbilical cord compression
d. Fetal sleep

N6 Normal baseline FHR is: NCLEX

a. 90–110 bpm
b. 110–160 bpm
c. 160–180 bpm
d. 60–100 bpm

N7 Daily fetal movement counting suggests compromise when the mother feels: NCLEX

a. 10 movements in 2 hours
b. Fewer than 10 movements in 12 hours
c. Many movements in the evening
d. Hiccups

N8 Before a contraction stress test, the nurse explains contractions will be produced by: NCLEX

a. Leopold's maneuvers
b. Ambulation
c. Small doses of oxytocin or nipple stimulation
d. Amniotomy

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Ideal time for fetal surveillance	After 24 weeks (viability in Egypt)	**Fetal movement count**	Daily from 28 wk; <10 in 12 h = compromise
NST timing	After 32 weeks, twice weekly	**Reactive NST**	≥2 accelerations ≥15 bpm × ≥15 s in 20 min → normal
BPP score	8–10 normal; ≤6 fetal compromise	**BPP components**	Breathing, movement, tone, amniotic fluid + NST
CST	Contractions induced by oxytocin or nipple stimulation	**Baseline FHR**	110–160 bpm
Bradycardia	Baseline < 110 bpm	**Acceleration**	Abrupt rise ≥15 bpm for ≥15 s — reassuring
Early deceleration	Head compression — mirrors contraction	**Late deceleration**	Uteroplacental insufficiency — after the peak
Variable deceleration	Cord compression — change position	**Amniotic fluid normal**	Clear/pale straw with white vernix specks
FHR 174 bpm	Tachycardia → notify provider		

Answer Key

الإجابات

1. a — Normal

2. b — Amniotic fluid volume and fetal activity

3. a — True

4. a — The uterus is stimulated to contract by small amounts of oxytocin or nipple stimulation

5. c — Notify the health-care provider

6. a — Inform the mother that the rate is normal

7. a — Inform the mother that the rate is normal

8. b — False

9. b — False

10. a — True

11. a — True

12. a — Uterine contractions are stimulated by small amounts of oxytocin

13. b — Notify the health care provider

14. d — Surrounds, cushions and protects the fetus

15. a — Administer oxygen via face mask

16. d — Reposition the catheter, recheck, and if it is 55% keep monitoring

17. c — Clear, almost colorless, with little white specks

18. b — Change the client's position

19. c — Below the umbilicus on the right side

20. a — An acceleration

21. c — So that each fetal heart rate is monitored separately

22. b — Assessing the baseline fetal heart rate

23. b — Variable decelerations

24. d — Notify the physician or nurse-midwife

25. a — Document the findings and tell the mother the monitor indicates fetal well-being

26. d — Palpating the maternal radial pulse while listening to the fetal heart rate

27. b — The ultrasound shows the blood flow through the umbilical cord
Ultrasound before amniocentesis locates the placenta, fetus and amniotic fluid pockets; it is not done to show cord blood flow.

28. c — 150 bpm

N1. b — Reactive (reassuring)
Reactive NST = ≥2 accelerations ≥15 bpm × ≥15 s within 20 min.

N2. b — Fetal compromise — notify provider
8–10 normal; 6 or below indicates fetal compromise.

N3. b — Turn the woman to her left side
Intrauterine resuscitation: lateral position, stop oxytocin, oxygen, IV fluids, notify.

N4. a, b, c, d, e — Fetal breathing movements; Fetal body movements; Fetal tone; Amniotic fluid volume; Reactive NST
BPP = 4 ultrasound variables + NST; maternal BP is not included.

N5. c — Umbilical cord compression
Variable = cord compression; early = head compression; late = placental insufficiency.

N6. b — 110–160 bpm
Below 110 is bradycardia; above 160 is tachycardia.

N7. b — Fewer than 10 movements in 12 hours
<10 kicks in 12 hours (or <3 per hour) needs evaluation.

N8. c — Small doses of oxytocin or nipple stimulation
CST evaluates FHR response to contractions induced by oxytocin or nipple stimulation.